



DISCHARGE SUMMARY

Department of General Surgery

Patient Name: Rajesh Sharma

Age / Sex: 46 Y / Male

Date of Admission: 12/01/2026

Consultant: Dr. Anil Mehta, MS (General Surgery)

Insurer: Demo Health Insurance

UHID: UHID-123456

IPD No.: IPD/2026/004512

Date of Discharge: 16/01/2026

Ward / Room: Surgical Ward - Room 304 (Twin Sharing)

TPA: Demo TPA

DIAGNOSIS

Final Diagnosis: Acute cholecystitis

ICD-10: K81.0

PROCEDURE

Procedure: Laparoscopic Cholecystectomy

Anaesthesia: General anaesthesia

Anaesthetist: Dr. Priya Nair

Date of Surgery: 13/01/2026

Surgeon: Dr. Anil Mehta

Outcome: Uneventful

PRESENTING COMPLAINTS

- Right upper abdominal pain for 2 days.
- Fever with chills for 1 day.
- Nausea and vomiting.

FINDINGS ON ADMISSION

Temperature 101.2 °F, pulse 104 / min, BP 128 / 82 mmHg. Tenderness in the right hypochondrium with a positive Murphy's sign. No icterus. Systemic examination otherwise unremarkable.

PAST HISTORY

Recurrent post-prandial abdominal pain for 3 months. No diabetes, hypertension or previous surgery. No known drug allergies.



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INVESTIGATIONS

Test	12/01/2026	15/01/2026	Reference range
Haemoglobin (g/dl)	13.4	12.9	13.0 - 17.0
Total leucocyte count (/cu mm)	14,200	8,900	4,000 - 11,000
Platelet count (lakh /cu mm)	2.4	2.6	1.5 - 4.1
Total bilirubin (mg/dl)	1.4	0.9	0.3 - 1.2
SGOT / AST (U/L)	48	34	< 40
SGPT / ALT (U/L)	52	38	< 41
Alkaline phosphatase (U/L)	138	112	40 - 129
Serum amylase (U/L)	62	-	28 - 100
Serum creatinine (mg/dl)	0.9	0.8	0.7 - 1.3

IMAGING

USG whole abdomen (12/01/2026): distended gall bladder with multiple calculi (largest 14 mm), wall thickening of 5 mm and pericholecystic fluid; sonographic Murphy's sign positive; CBD 5 mm, not dilated. Impression: acute calculous cholecystitis.

COURSE IN HOSPITAL

The patient was admitted on 12/01/2026 with features of acute cholecystitis and started on IV antibiotics, analgesics and IV fluids. After pre-anaesthetic evaluation and informed consent, he underwent Laparoscopic Cholecystectomy under general anaesthesia on 13/01/2026. Intra-operatively the gall bladder was distended and inflamed, with multiple calculi and omental adhesions. The specimen was sent for histopathological examination.

The post-operative period was uneventful. The drain was removed on post-operative day 2. He was mobilised early, tolerated an oral diet and remained afebrile with a healthy wound. He is being discharged in a stable condition.



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CONDITION AT DISCHARGE

General Condition: Stable, afebrile

Wound: Healthy, dressing dry

Vitals: Pulse 82 / min, BP 118 / 76 mmHg

Diet: Tolerating soft diet

DISCHARGE MEDICATIONS

Medicine	Dose	Frequency	Duration
Tab. Cefuroxime Axetil 500 mg	1 tablet	Twice daily after food	5 days
Tab. Paracetamol 650 mg	1 tablet	When required for pain (max 3 / day)	5 days
Tab. Pantoprazole 40 mg	1 tablet	Once daily before breakfast	7 days
Tab. Ondansetron 4 mg	1 tablet	When required for nausea	3 days
Syp. Lactulose	15 ml	At bedtime if constipated	5 days

ADVICE

- Soft, low-fat diet for 2 weeks; plenty of oral fluids.
- Keep the wounds clean and dry; dressing change on 19/01/2026.
- Avoid lifting heavy weights for 4 weeks.
- Walk regularly and resume routine activities gradually.

FOLLOW-UP

Review in the General Surgery OPD with Dr. Anil Mehta on 23/01/2026 with the histopathology report.

RETURN TO HOSPITAL IMMEDIATELY IF

- Fever above 100 °F, increasing abdominal pain or vomiting.
- Yellowish discolouration of the eyes or urine.
- Discharge or bleeding from the wounds.

DR. ANIL MEHTA
MS (GENERAL SURGERY)
REG. DEMO-MC-20417

Treating Consultant

Dr. Anil Mehta, MS (General Surgery)
Date: 16/01/2026

Prepared by

Dr. Kunal Joshi, Resident (General Surgery)
Date: 16/01/2026